

Patient NAME : Ms Kusuma Suresh
 DOB/Age/Gender : 64 Y/Female
 Patient ID / UHID : 14249036/RCL13942979
 Referred BY : Self
 Sample Collected : Oct 21, 2025, 07:48 AM

Report STATUS : Final Report
 Barcode NO : 27272714
 Sample Type : Whole blood EDTA
 Report Date : Oct 21, 2025, 02:39 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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24HI-1135-HC Package 1 With PPBS

Complete Blood Count (CBC)

RBC Parameters			
Hemoglobin <i>colorimetric</i>	13.6	g/dL	12.0 - 15.0
RBC Count <i>Electrical impedance</i>	4.47	10 ⁶ /μl	3.8 - 4.8
PCV <i>Calculated</i>	40	%	36 - 46
MCV <i>Calculated</i>	89.5	fl	83 - 101
MCH <i>Calculated</i>	30.5	pg	27 - 32
MCHC <i>Calculated</i>	34.1	g/dL	31.5 - 34.5
RDW (CV) * <i>Calculated</i>	11.4	%	11.6 - 14.0
RDW-SD * <i>Calculated</i>	38.7	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance and microscopy</i>	6.35	10 ³ /μl	4 - 10
Differential Leucocyte Count			
Neutrophils	53.7	%	40-80
Lymphocytes	37.7	%	20-40
Monocytes	6.6	%	2-10
Eosinophils	1	%	1-6
Basophils	1	%	<2
Absolute Leukocyte Counts <i>Calculated</i>			
Neutrophils.	3.41	10 ³ /μl	2 - 7
Lymphocytes.	2.39	10 ³ /μl	1 - 3
Monocytes.	0.42	10 ³ /μl	0.2 - 1.0
Eosinophils.	0.06	10 ³ /μl	0.02 - 0.5
Basophils.	0.06	10 ³ /μl	0.02 - 0.5
Platelet Parameters			
Platelet Count <i>Electrical impedance and microscopy</i>	295	10 ³ /μl	150 - 410
Mean Platelet Volume (MPV) * <i>Calculated</i>	7.8	fL	9.3 - 12.1
PCT * <i>Calculated</i>	0.2	%	0.17 - 0.32

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Test Description	Value(s)	Unit(s)	Reference Range
PDW * <i>Calculated</i>	9.9	fL	8.3 - 25.0
P-LCR * <i>Calculated</i>	13	%	18 - 50
P-LCC * <i>Calculated</i>	38	10 ⁹ /L	44 - 140
Mentzer Index * <i>Calculated</i>	20.02	%	> 13

Interpretation:

CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate MODIFIED WESTERGREN	10	mm/hr	0 - 14
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Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

Reference- Dacie and Lewis practical hematology

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Test Description	Value(s)	Unit(s)	Reference Range
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HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) HPLC	6.1	%	<5.7
Estimated Average Glucose *	128.37		

Interpretation:**Interpretation For HbA1c% As per American Diabetes Association (ADA)**

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

- Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
- Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

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Report STATUS : Final Report

Barcode NO : 23805677

Sample Type : FLUORIDE F

Report Date : Oct 21, 2025, 03:55 PM.

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Test Description	Value(s)	Unit(s)	Reference Range
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Glucose Fasting

Glucose Fasting <i>Hexokinase</i>	88.4	mg/dL	70 - 100
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Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association**Comment :**

Blood glucose determinations are commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy, insulinoma, or various liver diseases.

Note

1. The diagnosis of Diabetes requires a fasting plasma glucose of $\geq$ 126 mg/dL or a random / 2 hour plasma glucose value of $\geq$ 200 mg/dL with symptoms of diabetes mellitus.
2. Very high glucose levels ($>$ 450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Sample Type : Serum

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Bilirubin (Total, Direct, Indirect)

Bilirubin Total <i>Photometric</i>	0.41	mg/dL	0.2 - 1.2
Bilirubin Direct * <i>Diazo Reaction</i>	0.2	mg/dL	0.0 - 0.5
Bilirubin Indirect * <i>Calculation (T Bil - D Bil)</i>	0.21	mg/dL	0.1 - 1.0

Interpretation:

Adults and children

Increased total bilirubin that is mainly unconjugated (indirect) bilirubin may be a result of:-

1. Hemolytic or pernicious anemia
2. Transfusion reaction
3. Cirrhosis
4. A relatively common inherited condition called Gilbert syndrome, due to low levels of the enzyme that produces conjugated bilirubin.

Newborns

An elevated bilirubin level in a newborn may be temporary and resolve itself within a few days to two weeks. However, if the bilirubin level is above a critical threshold or increases rapidly, an investigation of the cause is needed so appropriate treatment can be initiated. Increased bilirubin concentrations may result from the accelerated breakdown of red blood cells due to:

1. Blood type incompatibility between the mother and her newborn
2. Certain congenital infections
3. Lack of oxygen (hypoxia)
4. Diseases that can affect the liver

In most of these conditions, only unconjugated (indirect) bilirubin is increased.

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SGOT/SGPT

SGOT/AST IFCC with P5P	18.9	U/L	0 - 35
SGPT/ALT IFCC with P5P	17.8	U/L	0 - 35
SGOT/SGPT Ratio *	1.06	-	-

Interpretation:

SGOT/SGPT ratio results help in the following interpretation:

Ratio < 1 : indicates non-alcoholic fatty liver disease.

Ratio = 1 : indicates acute viral hepatitis or drug-related liver toxicity.

Ratio > :1 indicates cirrhosis. A

ratio > 2:1 indicates alcoholic liver disease.

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Lipid Profile

Total Cholesterol <i>Enzymatic - Cholesterol Oxidase</i>	211	mg/dL	<200
Triglycerides <i>Colorimetric - Lip/Glycerol Kinase</i>	135	mg/dL	<150
HDL Cholesterol <i>Accelerator Selective Detergent</i>	44.4	mg/dL	>40
Non HDL Cholesterol * <i>Calculated</i>	166.6	mg/dL	<130
LDL Cholesterol * <i>Calculated</i>	139.6	mg/dL	<100
V.L.D.L Cholesterol * <i>Calculated</i>	27	mg/dL	< 30
Chol/HDL Ratio * <i>Calculated</i>	4.75	Ratio	3.5 - 5.0
HDL/ LDL Ratio * <i>Calculated</i>	0.32	Ratio	0.5 - 3.0
LDL/HDL Ratio * <i>Calculated</i>	3.14	Ratio	-

Interpretation:

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence

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Test Description		Value(s)	Unit(s)	Reference Range
High Risk	of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque			
Moderate Risk	2 major ASCVD risk factors			
Low Risk	0-1 major ASCVD risk factors			
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors				
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use			
2. Family history of premature ASCVD	4. High blood pressure			
5. Low HDL				

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

* After an adequate non-pharmacological intervention for at least 3 months.

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

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Iron Studies

Iron <i>Ferene</i>	58.4	µg/dL	50 - 170
TIBC,(Total Iron Binding Capacity) <i>Calculated</i>	306.4	µg/dL	250 - 450
UIBC <i>Ferene</i>	248	µg/dL	70 - 310
Transferrin Saturation <i>Method :Derived from IRON and TIBC values</i>	19.06	%	-

Interpretation:

Increased levels due to iron ingestion or ineffective erythropoiesis. Decreased levels due to infection, inflammation, malignancy, menstruation and Fe deficiency. Needs to be taken into consideration with TIBC. Transferrin Saturation:- Low level Transferrin Saturation can indicate iron deficiency, erythropoiesis, infection, or inflammation. High level Transferrin Saturation can indicate recent ingestion of dietary iron, ineffective erythropoiesis, haemochromatosis or liver disease. High TIBC, UIBC, or transferrin usually indicates iron deficiency, but they are also increased in pregnancy and with the use of oral contraceptives. Low TIBC, UIBC, or transferrin may occur if someone has: Hemochromatosis, Certain types of anemia due to accumulated iron, Malnutrition, kidney disease that causes a loss of protein in urine.

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High Sensitivity C-Reactive Protein (Hs-CRP)

HIGHLY SENSITIVE C-REACTIVE PROTEIN (hs-CRP) <i>immunoturbidimetric</i>	2.3	mg/L	< 1.00
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Interpretation:

Cardio CRP In mg/L	Cardiovascular Risk
<1	Low
1-3	Average
3-10	High
>10	Persistent elevation may represent Non cardiovascular inflammation

Note: To assess vascular risk, it is recommended to test hsCRP levels 2 or more weeks apart and calculate the average**Comments:**

High sensitivity C Reactive Protein (hsCRP) significantly improves cardiovascular risk assessment as it is a strongest predictor of future coronary events. It reveals the risk of future Myocardial infarction and Stroke among healthy men and women, independent of traditional risk factors. It identifies patients at risk of first Myocardial infarction even with low to moderate lipid levels. The risk of recurrent cardiovascular events also correlates well with hsCRP levels. It is a powerful independent risk determinant in the prediction of incident Diabetes.

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Vitamin B12 / Cyanocobalamin

Vitamin - B12 ECLIA	557	pg/mL	187 - 883
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Interpretation:

Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms.

Causes of vitamin B12 deficiency include: Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

1. Out of 140 healthy indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."

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Patient NAME : Ms Kusuma Suresh

DOB/Age/Gender : 64 Y/Female

Patient ID / UHID : 14249036/RCL13942979

Referred BY : Self

Sample Collected : Oct 21, 2025, 07:48 AM

Report STATUS : Final Report

Barcode NO : 27322334

Sample Type : Serum

Report Date : Oct 21, 2025, 03:55 PM.

Test Description	Value(s)	Unit(s)	Reference Range
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Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy ECLIA	20.9	ng/mL	Deficiency : <30 ng/mL
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Interpretation:

25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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Dr. Lakshmi Kasula
MD (Path)
Consultant Pathologist



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Processing Lab :- Redcliffe Lifetech Pvt. Ltd., 2nd FLOOR, 1-4-168/4/4/NR, Satya Complex Loyola College Road, Landmark Karur Vysya Bank, Old Alwal Secunderabad-500010,

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Thyroid Profile Total

Triiodothyronine (T3) ECLIA	116	ng/dL	80 - 200
Total Thyroxine (T4) ECLIA	7.51	µg/dL	4.5 - 11.7
Thyroid Stimulating Hormone (Ultrasensitive) CMIA	2.0	mIU/L	0.35 - 4.94

Interpretation:

Pregnancy	Reference Range TSH
1st Trimester	0.1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Clinical Use:

1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance**Decreased TSH:** Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. **Primary Hypothyroidism:** High TSH levels.
2. **Secondary/Tertiary Hypothyroidism:** Low TSH levels.
3. **Euthyroid Sick Syndrome:** Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Normal	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

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Test Description	Value(s)	Unit(s)	Reference Range
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Phosphorus

Phosphorus Photometric	3.7	mg/dL	2.3 - 4.7
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Interpretation:

Increased levels are found in acute myocardial infarction (AMI), AMI with congestive heart failure, cirrhosis, obstructive jaundice, hemolytic anemia, various malignancies, skeletal muscle injury, electric and thermal burns.

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Test Description	Value(s)	Unit(s)	Reference Range
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Kidney Function Test (KFT/RFT)

Blood Urea <i>Urease</i>	22.8	mg/dL	18 - 55
Bun * <i>Urease</i>	10.65	mg/dL	9.8 - 20.1
Creatinine <i>Jaffe Kinetic</i>	0.9	mg/dl	0.50 - 0.90
eGFR (CKD-EPI)	71.37	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio * <i>Calculated</i>	11.83		12 - 20
Urea / Creatinine Ratio * <i>Calculated</i>	25.33		25.68- 42.8
Uric Acid <i>Uricase</i>	4.7	mg/dL	2.4 - 5.7
Calcium Serum <i>Arsenazo III</i>	9.6	mg/dL	8.8 - 10.0

Interpretation:

Kidney function tests is a collective term for a variety of individual tests and procedures that can be done to evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carry out their vital functions. Some lead to a rapid (acute) decline in kidney function, others lead to a gradual (chronic) decline in function. Both result in a buildup of toxic waste substances done on urine samples, as well as on blood samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure, blood in urine, frequent urges to urinate, difficulty beginning urination, painful urination, swelling in the hands and feet due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Electrolytes are present in the human body and the balancing act of the electrolytes in our bodies is essential for normal function of our cells and organs. There has to be a balance. Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases.

"eGFR test is applicable for patients aged 18 years or more."

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 Patient ID / UHID : 14249036/RCL13942979
 Referred BY : Self
 Sample Collected : Oct 21, 2025, 07:51 AM

Report STATUS : Final Report

Barcode NO : 28711739

Sample Type : Spot Urine

Report Date : Oct 21, 2025, 03:01 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Urine Routine and Microscopic Examination

Physical Examination *			
Volume *	15	ml	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	Clear	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination *			
Reaction (pH) <i>Double Indicator</i>	6.5	-	4.5 - 8.0
Specific Gravity <i>Ion Exchange</i>	1.010	-	1.010 - 1.030
Urine Glucose (sugar) <i>Oxidase / Peroxidase</i>	Negative	-	Negative
Urine Protein (Albumin) <i>Acid / Base Colour Exchahnge</i>	Negative	-	Negative
Urine Ketones (Acetone) <i>Legals Test</i>	Negative	-	Negative
Blood <i>Peroxidase Hemoglobin</i>	Negative	-	Negative
Leucocyte esterase <i>Enzymatic Reaction</i>	Negative	-	Negative
Bilirubin Urine <i>Coupling Reaction</i>	Negative	-	Negative
Nitrite <i>Griless Test</i>	Negative	-	Negative
Urobilinogen <i>Ehrlichs Test</i>	Normal	-	Normal
Microscopic Examination *			
Pus Cells (WBCs) *	2-4	/hpf	0 - 5
Epithelial Cells *	3-5	/hpf	0 - 4
Red blood Cells *	Absent	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	Absent	-	Absent
Protozoa *	Absent	-	Absent

Interpretation:**URINALYSIS-** Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.**Protein:** Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

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Test Description	Value(s)	Unit(s)	Reference Range
Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications. Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise. Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders. Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection. Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection. pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine. Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus. Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine. Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.			

*** End Of Report ***

Pending Report To Follow - Glu-PP

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Bio-Rad CDM System
VII Inst. #1. SN-16442

PATIENT REPORT
V2TURBO_A1c_2.0

Patient Data

Sample ID: 27272714
 Patient ID:
 Name:
 Physician:
 Sex:
 DOB:

Analysis Data

Analysis Performed: 10/21/2025 16:06:16
 Injection Number: 4623
 Run Number: 89
 Rack ID: 0006
 Tube Number: 5
 Report Generated: 10/21/2025 16:15:19
 Operator ID:

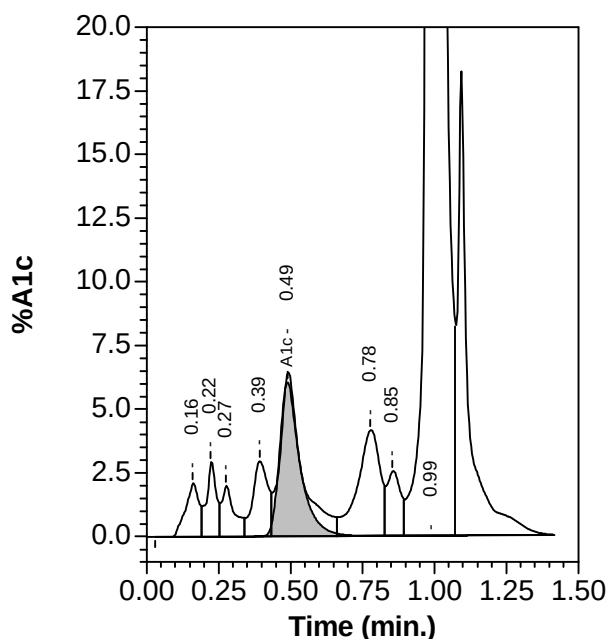
Comments:

Peak Name	NGSP %	Area %	Retention Time (min)	Peak Area
A1a	---	1.1	0.158	23733
A1b	---	1.2	0.221	24436
F	---	1.0	0.274	21440
LA1c	---	1.7	0.392	36699
A1c	6.1*	---	0.488	104786
P3	---	3.8	0.777	78786
P4	---	1.4	0.853	28715
Ao	---	84.8	0.988	1780093

*Values outside of expected ranges

Total Area: 2,098,689

HbA1c (NGSP) = 6.1* %



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2. It is to be presumed that the tests performed pertain to the specimen/sample attributed to the Customer's name or identification. It is presumed that the verification particulars have been cleared out by the customer or his/her representation at the point of generation of said specimen / sample. It is hereby clarified that the reports furnished are restricted solely to the given specimen only.
3. It is to be noted that variations in results may occur between different laboratories and over time, even for the same parameter for the same Customer. The assays are performed and conducted in accordance with standard procedures, and the reported outcomes are contingent on the specific individual assay methods and equipment(s) used, as well as the quality of the received specimen.
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We value your feedback, please scan with your phone camera
and rate overall experience.

How likely are you to recommend Redcliffe Labs to your family & friends

1	2	3	4	5	6	7	8	9	10
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Not Likely

 Very Likely



Scan with your phone camera